

SUMMARY OF PRODUCT CHARACTERISTICS
VIRANZ 600 (Efavirenz Tablets USP 600 mg)

Rx Only

1. NAME OF THE FINISHED PHARMACEUTICAL PRODUCT:

Efavirenz Tablets USP 600 mg

(TRADE) NAME OF THE PRODUCT: VIRANZ

STRENGTH: 600 mg

2. QUALITATIVE AND QUANTITATIVE COMPOSITIONS:

Each film - coated tablet contains Efavirenz USP 600 mg.

Excipients:

Tablets core: Microcrystalline cellulose, sodium lauryl sulfate, croscarmellose sodium, hydroxypropyl cellulose, lactose monohydrate and magnesium stearate.

Film coat: Hypromellose, titanium dioxide, polyethylene glycol and yellow iron oxide

Description:

Yellow coloured oval biconvex film coated tablets with 'D' on one side and '37' on the other side.

3. INDICATIONS AND USAGE:

VIRANZ 600 (efavirenz) in combination with other antiretroviral agents is indicated for the treatment of human immunodeficiency virus type 1 (HIV-1) infection in adults and in pediatric patients at least 3 months old and weighing at least 3.5 kg.

4. CLINICAL PHARMACOLOGY

4.1 Mechanism of Action:

Efavirenz is a selective non-nucleoside reverse transcriptase inhibitor of human immunodeficiency virus type 1 (HIV-1). Efavirenz is a non-competitive inhibitor of HIV-1 reverse transcriptase (RT) with respect to template, primer or nucleoside triphosphates, with a small component of competitive inhibition. HIV-2 RT and human cellular DNA polymerases α , β , γ and δ are not inhibited by concentrations of efavirenz well in excess of those achieved clinically.

4.2 Pharmacokinetics

Absorption

Peak efavirenz plasma concentrations of 1.6-9.1 μM were attained by 5 hours following single oral doses of 100 mg to 1600 mg administered to uninfected volunteers. Dose-related increases in C_{max} and AUC were seen for doses up to 1600 mg; the increases were less than proportional suggesting diminished absorption at higher doses. Time to peak plasma concentrations (3-5

hours) did not change following multiple dosing and steady-state plasma concentrations were reached in 6-7 days.

In HIV-infected patients at steady-state, mean C_{max} , mean C_{min} , and mean AUC were linear with 200 mg, 400 mg, and 600 mg daily doses. In 35 patients receiving VIRANZ 600 mg once daily, steady-state C_{max} was 12.9 μ M, steady-state C_{min} was 5.6 μ M, and AUC was 184 μ Mh.

Effect of Food on Oral Absorption

The bioavailability of a single 600-mg dose of efavirenz in uninfected volunteers was increased 22% and 17%, respectively, when given with a meal of high fat or normal composition, relative to the bioavailability of a 600-mg dose given under fasted conditions. VIRANZ 600 can be administered with or without food.

Distribution

Efavirenz is highly bound (approximately 99.5-99.75%) to human plasma proteins, predominantly albumin. In HIV-1 infected patients (N=9) who received VIRANZ 200 to 600 mg once daily for at least one month, cerebrospinal fluid concentrations ranged from 0.26 to 1.19% (mean 0.69%) of the corresponding plasma concentration. This proportion is approximately 3-fold higher than the non-protein-bound (free) fraction of efavirenz in plasma.

Metabolism

Studies in humans and in vitro studies using human liver microsomes have demonstrated that efavirenz is principally metabolized by the cytochrome P450 system to hydroxylated metabolites with subsequent glucuronidation of these hydroxylated metabolites. These metabolites are essentially inactive against HIV-1. The in vitro studies suggest that CYP3A4 and CYP2B6 are the major isozymes responsible for efavirenz metabolism. In vitro studies have shown that efavirenz inhibited P450 isozymes 2C9, 2C19, and 3A4 with K_i values (8.5-17 μ M) in the range of observed efavirenz plasma concentrations. In in vitro studies, efavirenz did not inhibit CYP2E1 and inhibited CYP2D6 and CYP1A2 (K_i values 82-160 μ M) only at concentrations well above those achieved clinically.

Efavirenz plasma exposure may be increased in patients with the homozygous G516T genetic variant of the CYP2B6 isoenzyme. The clinical implications of such an association are unknown; however, the potential for an increased frequency and severity of efavirenz associated adverse events cannot be excluded.

Efavirenz has been shown to induce P450 enzymes, resulting in the induction of its own metabolism. Multiple doses of 200-400 mg per day for 10 days resulted in a lower than predicted extent of accumulation (22-42% lower) and a shorter terminal half-life of 40-55 hours (single dose half-life 52-76 hours). The degree of CYP3A4 induction is expected to be similar between a 400-mg and 600-mg dose of efavirenz based on pharmacokinetic interaction studies in which daily 400-mg or 600-mg efavirenz doses in combination with indinavir did not appear to cause any further reduction of indinavir AUC compared to a 200 mg dose of efavirenz.

Elimination

Efavirenz has a relatively long terminal half-life of 52 to 76 hours after single doses and 40-55 hours after multiple doses. Approximately 14-34% of a radiolabeled dose of efavirenz was recovered in the urine and less than 1% of the dose was excreted in urine as unchanged efavirenz.

4.3 Pharmacodynamics

Viranz 600 is a selective non-nucleoside reverse transcriptase inhibitor (NNRTI) of human immunodeficiency virus type 1 (HIV-1). Viranz 600 is a non-competitive inhibitor of HIV-1 reverse transcriptase (RT) with respect to template, primer or nucleoside triphosphates. with a small component of competitive inhibition. HIV-2 RT and human cellular DNA polymerases alpha, beta, gama and omega are not inhibited by concentrations of Viranz 600 well in excess of those achieved clinically.

Cardiac Electrophysiology

The effect of efavirenz tablets 600mg on the QTc interval was evaluated in an open-label, positive and placebo controlled, fixed single sequence 3-period, 3-treatment crossover QT study in 58 healthy subjects enriched for CYP2B6 polymorphisms. The mean C_{max} of efavirenz in subjects with CYP2B6 *6/*6 genotype following the administration of 600 mg daily dose for 14 days was 2.25-fold the mean C_{max} observed in subjects with CYP2B6 *1/*1 genotype. A positive relationship between efavirenz concentration and QTc prolongation was observed. Based on the concentration-QTc relationship, the mean QTc prolongation and its upper bound 90% confidence interval are 8.7 ms and 11.3 ms in subjects with CYP2B6*6/*6 genotype following the administration of 600 mg daily dose for 14 days. (see Section Warnings and Precautions & Section Interaction with Other Medicaments).

5. DOSAGE AND ADMINISTRATION

Hepatic Function

Monitor hepatic function prior to and during treatment with VIRANZ 600 [see Warnings and Precautions].

VIRANZ 600 is not recommended in patients with moderate or severe hepatic impairment (Child Pugh B or C) [See Warnings and Precautions].

Adults

The recommended dosage of VIRANZ (efavirenz) is 600 mg orally, once daily, in combination with a protease inhibitor and/or nucleoside analogue reverse transcriptase inhibitors (NRTIs). It is recommended that VIRANZ 600 be taken on an empty stomach, preferably at bedtime. The increased efavirenz concentrations observed following administration of VIRANZ 600 with food may lead to an increase in frequency of adverse reactions [see Clinical Pharmacology]. Dosing at bedtime may improve the tolerability of nervous system symptoms

Dosage Adjustment

If VIRANZ 600 is coadministered with voriconazole, the voriconazole maintenance dose should be increased to 400 mg every 12 hours and the VIRANZ 600 dose should be decreased to 300 mg once daily. Viranz 600 tablets must not be broken. [See Interactions with other Medicaments.] If VIRANZ 600 is coadministered with rifampin to patients weighing 50 kg or more, an increase in the dose of VIRANZ to 800 mg once daily is recommended [See Interactions with other Medicaments].

Renal impairment

The pharmacokinetics of efavirenz have not been studied in patients with renal insufficiency; however, less than 1% of an efavirenz dose is excreted unchanged in the urine, so the impact of renal impairment on efavirenz elimination should be minimal (see section Warnings and Precautions).

Pediatric Patients

It is recommended that VIRANZ 600 be taken on an empty stomach, preferably at bedtime. Table 1 describes the recommended dose of VIRANZ 600 for pediatric patients 3 months of age or older and weighing between 3.5 kg and 40 kg [see Clinical Pharmacology]. The recommended dosage of VIRANZ 600 for pediatric patients weighing 40 kg or greater is 600 mg once daily.

Table 1

Pediatric Dose to be administered once daily

Body weight, kg	Viranz capsules and tablets Dose (mg)
13 to <15	200
15 to <20	250
20 to <25	300
25 to <32.5	350
32.5 to <40	400
≥40	600

6. CONTRAINDICATIONS

VIRANZ 600 is contraindicated in patients with clinically significant hypersensitivity to any of its components.

VIRANZ 600 is contraindicated with elbasvir/grazoprevir due to the expected significant decreases in elbasvir and grazoprevir plasma concentrations (see DRUG INTERACTIONS). This effect is due to an induction of CYP3A4 by efavirenz and is expected to result in the loss of virologic response of elbasvir/grazoprevir.

VIRANZ 600 should not be administered concurrently with terfenadine, astemizole, cisapride, midazolam, triazolam, pimozide, bepridil, or ergot derivatives because competition for CYP3A4 by efavirenz could result in inhibition of metabolism of these drugs and create the

potential for serious and/or life-threatening adverse events [e.g., cardiac arrhythmias, prolonged sedation, or respiratory depression].

VIRANZ 600 must not be administered concurrently with the standard doses of voriconazole because efavirenz significantly decreases voriconazole plasma concentrations while voriconazole also significantly increases efavirenz plasma concentrations (see DRUG INTERACTIONS; for use of adjusted doses of voriconazole with adjusted doses of efavirenz, see DRUG INTERACTIONS).

St. John's wort (*Hypericum perforatum*): Patients on efavirenz should not concomitantly use products containing St. John's wort (*Hypericum perforatum*) since it may be expected to result in reduced plasma concentrations of efavirenz. This effect is due to an induction of CYP3A4 and may result in loss of therapeutic effect and development of resistance.

7. WARNINGS AND PRECAUTIONS

VIRANZ 600 must not be used as a single agent to treat HIV or added on as a sole agent to a failing regimen.

When prescribing drugs concomitantly with VIRANZ 600, physicians should refer to the corresponding manufacturer's product circular.

If any antiretroviral medication in a combination regimen is interrupted because of suspected intolerance, serious consideration should be given to simultaneous discontinuation of all antiretroviral medications. The antiretroviral medications should be restarted at the same time upon resolution of the intolerance symptoms. Intermittent monotherapy and sequential reintroduction of antiretroviral agents is not advisable because of the increased potential for selection of drug-resistant mutant virus.

Malformations have been observed in foetuses from efavirenz-treated animals (see Use in pregnancy); therefore, pregnancy should be avoided in women receiving VIRANZ 600. Barrier contraception should always be used in combination with other methods of contraception (eg, oral or other hormonal contraceptives).

Skin Rash: Mild-to-moderate rash has been reported with Efavirenz and usually resolves with continued therapy. Appropriate antihistamines and/or corticosteroids may improve the tolerability and hasten the resolution of rash. Severe rash associated with blistering, moist desquamation or ulceration has been reported in <1% of patients treated with Efavirenz. The incidence of erythema multiforme or Stevens-Johnson syndrome was also observed. Efavirenz should be discontinued in patients developing severe rash associated with blistering, desquamation, mucosal involvement or fever. If therapy with Efavirenz is discontinued, consideration should also be given to interrupting therapy with other antiretroviral agents to avoid development of drug resistant virus. Rash was reported in children treated with Efavirenz. Prophylaxis with appropriate antihistamines prior to initiating therapy with Efavirenz in children may be considered.

Psychiatric Symptoms: Psychiatric adverse experiences have been reported in patients treated with efavirenz. Patients with a history of psychiatric disorders appear to be at greater risk of these serious psychiatric adverse experiences. There have also been occasional post- marketing

reports of death by suicide, delusions and psychosis-like behaviour, although a causal relationship to the use of efavirenz cannot be determined from these reports. Patients should be advised that if they experience these symptoms, they should contact their doctor immediately to assess the possibility that the symptoms may be related to the use of efavirenz, and if so, to determine whether the risks of continued therapy outweigh the benefits.

Nervous System Symptoms:

Late-onset neurotoxicity, including ataxia and encephalopathy (impaired consciousness, confusion, psychomotor slowing, psychosis, delirium), may occur months to years after beginning efavirenz therapy. Some events of late-onset neurotoxicity have occurred in patients with CYP2B6 genetic polymorphisms, which are associated with increased efavirenz levels despite standard dosing of VIRANZ 600. Patients presenting with signs and symptoms of serious neurologic adverse experiences should be evaluated promptly to assess the possibility that these events may be related to efavirenz use, and whether discontinuation of VIRANZ 600 is warranted.

Seizures: Convulsions have been observed rarely in patients receiving efavirenz, generally in the presence of known medical history of seizures. Patients who are receiving concomitant anticonvulsant medicinal products primarily metabolized by the liver, such as phenytoin, carbamazepine and phenobarbital, may require periodic monitoring of plasma levels. Caution must be taken in any patient with a history of seizures.

Effect of Food: The administration of VIRANZ 600 with food may increase efavirenz exposure and may lead to an increase in the frequency of undesirable effects. This effect may be more evident for the tablets than for the hard capsules. Taking VIRANZ 600 on an empty stomach, preferably at bedtime, can be considered.

Immune Reconstitution Syndrome: Immune reconstitution syndrome has been reported in patients treated with combination antiretroviral therapy (CART), including VIRANZ 600. During the initial phase of treatment, a patient whose immune system responds to CART may mount an inflammatory response to indolent or residual opportunistic infections, which may necessitate further evaluation and treatment.

Special Populations: Because of the extensive cytochrome P450-mediated metabolism of efavirenz and limited clinical experience in patients with chronic liver disease, caution should be exercised in administering VIRANZ 600 to patients with liver disease.

The pharmacokinetics of efavirenz have not been studied in patients with renal insufficiency; however, less than 1% of an efavirenz dose is excreted unchanged in the urine, so the impact of renal impairment on efavirenz elimination should be minimal.

Insufficient numbers of elderly patients have been evaluated in clinical studies to determine whether they respond differently than younger patients.

Efavirenz have not been evaluated in children below 3 years of age or who weigh less than 13 kg. Evidence exists indicating that efavirenz may have altered pharmacokinetics in very young children. For this reason, efavirenz tablets or oral solution should not be given to children less than 3 years of age.

Liver Enzymes: In patients with known or suspected history of Hepatitis B or C infection and in patients treated with other medications associated with liver toxicity, monitoring of liver enzymes is recommended. In patients with persistent elevations of serum transaminases to greater than 5 times the upper limit of the normal range, the benefit of continued therapy with Efavirenz needs to be weighed against the unknown risks of significant liver toxicity.

QTc Prolongation: QTc prolongation has been observed with the use of efavirenz (see Section Pharmacodynamics and Section Interaction with Other Medicaments). Consider alternatives to VIRANZ 600 when coadministered with a drug with a known risk of Torsade de Pointes or when administered to patients at higher risk of Torsade de Pointes.

Lipids: Monitoring of lipid levels should be considered in patients treated with VIRANZ 600.

8. INTERACTIONS WITH OTHER MEDICAMENTS

Efavirenz is an inducer of CYP3A4 and CYP2B6. Other compounds that are substrates of CYP3A4 or CYP2B6 may have decreased plasma concentrations when coadministered with VIRANZ 600.

Concomitant Antiretroviral Agents

Amprenavir: While clinical significance of decreased amprenavir concentrations has not been established, it should be taken into consideration when choosing a regimen containing both efavirenz and amprenavir.

Fosamprenavir calcium: For coadministration with fosamprenavir and ritonavir, the prescribing information for fosamprenavir calcium should be consulted.

Atazanavir: Efavirenz decreases atazanavir exposure. Refer to the prescribing information for atazanavir guidance on coadministration with efavirenz.

Indinavir: The optimal dose of indinavir, when given in combination with Efavirenz, is not known. Increasing the indinavir dose to 1000mg every 8 hours does not compensate for the increased indinavir metabolism due to efavirenz.

Ritonavir: When Efavirenz and ritonavir was administered in uninfected volunteers the combination was not well tolerated and was associated with a higher frequency of adverse clinical experiences (e.g., dizziness, nausea, paraesthesia) and laboratory abnormalities (elevated liver enzymes). Monitoring of liver enzymes is recommended when VIRANZ 600 is used in combination with ritonavir.

Saquinavir: Use of VIRANZ 600 in combination with saquinavir as the sole protease inhibitor is not recommended.

Rifamycins: Rifamycins reduced Viranz 600 concentration in uninfected volunteers. The dose of Efavirenz should be increased to 800 mg/day when taken with rifampin. No dose adjustment of Rifampin is recommended when given with Efavirenz. Rifabutin had no significant effect on the pharmacokinetics of Efavirenz. The daily dose of rifabutin should be increased by 50%

when administered with Viranz 600 and that the rifabutin dose may be doubled for regimens in which rifabutin is given two or three times a week in combination with Viranz 600.

Clarithromycin: No dose adjustment of Viranz 600 is recommended when given with clarithromycin. Alternative to clarithromycin should be considered.

Voriconazole: Coadministration of efavirenz and voriconazole is contraindicated (see CONTRAINDICATIONS).

Itraconazole: Since no dose recommendation for itraconazole can be made, alternative antifungal treatment should be considered.

Lipid-lowering Agents

Coadministration of efavirenz with the HMG-CoA reductase inhibitors atorvastatin, pravastatin, or simvastatin has been shown to reduce the plasma concentration of the statin in uninfected volunteers. Cholesterol levels should be periodically monitored. Dosage adjustments of statins may be required. Coadministration of efavirenz with atorvastatin, pravastatin, or simvastatin did not affect efavirenz AUC or C_{max} values. No dosage adjustment is necessary for efavirenz.

Anticonvulsants

Carbamazepine: Carbamazepine plasma levels should be monitored periodically. There are no data with coadministration of higher doses of either medicinal product; therefore, no dose recommendation can be made, and alternative anticonvulsant treatment should be considered.

Other anticonvulsants: No data are available on the potential interactions of efavirenz with phenytoin, phenobarbital, or other anticonvulsants that are substrates of CYP450 isozymes. When efavirenz is administered concomitantly with these agents, there is a potential for reduction or increase in the plasma concentrations of each agent; therefore, periodic monitoring of plasma levels should be conducted. Clinically significant interactions would not be expected since vigabatrin and gabapentin are exclusively eliminated unchanged in the urine and would be unlikely to compete for the same metabolic enzymes and elimination pathways as efavirenz.

Oral Contraceptives: Because the potential interaction of Viranz 600 with oral contraceptives has not been fully characterized, a reliable method of barrier contraception should be used in addition to oral contraceptives.

Methadone: In HIV-infected IV drug users, co-administration of efavirenz with methadone resulted in decreased plasma levels of methadone and signs of opiate withdrawal. The methadone dose was increased to alleviate withdrawal symptoms. Patients should be monitored for signs of withdrawal and their methadone dose increased as required to alleviate withdrawal symptoms.

Antidepressants: No dose adjustments are necessary for either efavirenz or paroxetine when these drugs are coadministered. Sertraline did not significantly alter the pharmacokinetics of efavirenz. The dose of sertraline should be increased when administered with efavirenz to compensate for the induction of sertraline metabolism by efavirenz. Sertraline dose increases should be guided by clinical response.

Cetirizine: No dose adjustments are necessary for either efavirenz or cetirizine when these drugs are coadministered.

Lorazepam: No dose adjustment is necessary for either efavirenz or lorazepam when these drugs are coadministered.

Calcium channel blockers: No data are available on the potential interactions of efavirenz with other calcium channel blockers that are substrates of the CYP3A4 enzyme (eg, verapamil, felodipine, nifedipine, nifedipine, nifedipine, nifedipine). When efavirenz is administered concomitantly with one of these agents, there is a potential for reduction in the plasma concentrations of the calcium channel blocker. Dose adjustments should be guided by clinical response.

Cannabinoid Test Interaction: Efavirenz does not bind to cannabinoid receptors. False positive urine cannabinoid test results have been reported.

QT Prolonging Drugs:

There is limited information available on the potential for a pharmacodynamics interaction between VIRANZ 600 and drugs that prolong the QTc interval. QTc prolongation has been observed with the use of efavirenz (see Section Pharmacodynamics and Section Warnings and Precautions). Consider alternatives to VIRANZ 600 when coadministered with a drug with a known risk of Torsade de Pointes.

UNDESIRABLE EFFECTS

Efavirenz was generally well tolerated. In patients who received efavirenz in combination with protease inhibitors and/or NRTIs, the most frequently reported treatment-related undesirable effects were rash, dizziness, nausea, headache and fatigue. The most notable undesirable effects associated with efavirenz are rash and nervous system symptoms.

Other undesirable effects include: Allergic reaction, abnormal coordination, ataxia, confusion, stupor, vertigo, vomiting, diarrhoea, hepatitis, impaired concentration, insomnia, anxiety, abnormal dreams, somnolence, depression, abnormal thinking, agitation, amnesia, delirium, emotional lability, euphoria, hallucination and psychosis.

Additional undesirable effects reported in post-marketing surveillance include neurosis, paranoid reaction, cerebellar coordination and balance disturbances, convulsions, pruritus, abdominal pain, blurred vision, flushing, gynecomastia, hepatic failure, photoallergic dermatitis, pancreatitis, redistribution/accumulation of body fat in areas such as the back of the neck, breasts, abdomen, and retroperitoneum, tinnitus, and tremor.

A few of the post-marketing reports of hepatic failure, including cases in patients with no pre-existing hepatic disease or other identifiable risk factors, were characterized by a fulminant course, progressing in some cases to transplantation or death.

The type and frequency of undesirable effects in children were generally similar to that of adult patients with the exception that rash was reported more frequently in children and was more often of higher grade than in adults.

Rash: Skin rash was considered treatment-related in patients treated with Efavirenz. The incidence of erythema multiforme or Stevens-Johnson syndrome was also reported. Prophylaxis with appropriate antihistamines prior to initiating therapy with efavirenz in children may be considered. Rashes are usually mild-to-moderate maculopapular skin eruptions that occur within the first 2 weeks of initiating therapy with Efavirenz. In most patients, rash resolves with continuing therapy with Efavirenz within 1 month. Efavirenz can be re-initiated in patients interrupting therapy because of rash. Use of appropriate

antihistamines and/or corticosteroids is recommended when Efavirenz is restarted.

Psychiatric Symptoms: Serious psychiatric adverse experiences have been reported in patients treated with efavirenz. Symptoms include severe depression, suicidal ideation, nonfatal suicide attempts, aggressive behaviour, paranoid reactions and manic reactions. Patients with a history of psychiatric disorders appear to be at greater risk of these serious psychiatric adverse experiences, with the frequency of each of the previously mentioned events ranging from manic reactions to both severe depression and suicidal ideation. There have also been occasional reports of death by suicide, delusions and psychosis-like behavior, although a causal relationship to the use of efavirenz cannot be determined from these reports.

Nervous System Symptoms: Symptoms including, but not limited to, dizziness, insomnia, somnolence, impaired concentration, and abnormal dreaming are frequently reported side effects in patients receiving Efavirenz 600 mg daily. Nervous system symptoms usually begin during the first 1 or 2 days of therapy and generally resolve after the first 2-4 weeks. Dosing at bedtime improves the tolerability of these symptoms and is recommended during the first weeks of therapy and in patients who continue to experience these symptoms. Dose reduction or splitting the daily dose has not been shown to provide benefit and is not recommended.

Postmarketing experiences: encephalopathy

10. EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

Dizziness, impaired concentration, and drowsiness have been reported during treatment with Viranz 600. If you experience these symptoms, you should avoid potentially hazardous tasks such as driving or operating machinery.

11. PREGNANCY

There are no adequate and well-controlled studies of efavirenz in pregnant women.

Pregnancy should be avoided in women treated with efavirenz. Barrier contraception should always be used in combination with other methods of contraception (for example, oral or other hormonal contraceptives). Because of the long half-life of efavirenz, use of adequate contraceptive measures for 12 weeks after discontinuation of Efavirenz is recommended. Women of childbearing potential should undergo pregnancy testing before initiation of efavirenz. Efavirenz should not be used during pregnancy unless the potential benefit to the mother clearly outweighs the potential risk to the fetus and there are no other appropriate treatment options. If a woman takes efavirenz during the first trimester of pregnancy or becomes pregnant while taking efavirenz, she should be informed of the potential harm to the fetus.

12. NURSING MOTHERS

Efavirenz is secreted into the milk of lactating rats and efavirenz also has been shown to pass into human breast milk. It is recommended that mothers taking efavirenz do not breast-feed their infants. It is recommended that HIV-infected women do not breast-feed their infants under any circumstances in order to avoid transmission of HIV.

13. PEDIATRIC USE

VIRANZ 600 tablets or oral solution have not been studied in pediatric patients below 3 years of age or who weigh less than 13 kg (see DOSAGE AND ADMINISTRATION, WARNINGS AND PRECAUTIONS).

14. POST MARKETING EXPERIENCE: Encephalopathy

15. OVERDOSAGE

Some patients accidentally taking 600 mg twice daily have reported increased nervous system symptoms. One patient experienced involuntary muscle contractions.

Treatment of overdose with VIRANZ 600 should consist of general supportive measures, including monitoring of vital signs and observation of the patient's clinical status.

Administration of activated charcoal may be used to aid removal of unabsorbed drug. There is no specific antidote for overdose with VIRANZ 600. Since efavirenz is highly protein bound, dialysis is unlikely to significantly remove the drug from blood.

16. STORAGE CONDITIONS

Do not store above 30°C. Store in the original package.

17. SHELF LIFE

36 months

18. PRESENTATION

Viranz 600:30's tablets packed in round white opaque HDPE bottle.

Each such container will be packed in a monocarton with package insert.

19. MANUFACTURED BY:

Aurobindo Pharma Ltd. Unit-III Sy.No. 313 & 314 ,

Bachupally, Bachupally Mandal,

Medchal-Malkajgiri District,

Telangana state, INDIA.

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